Clinical Trials III

Case Studies

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Course content

1. Basic concepts related to CTs
2. Designing/Reporting CTs/Mendelian Randomisation/Survival Analysis
3. Reporting CTs (CONSORT guidelines)
4. Discussion on the controversial PACE trial
5. Project's Presentations/Course Summary

CONSORT

Consolidated Standard for Reporting Trials

1996

Begg C, Cho M, Eastwood S, et al. Improving the quality of reporting of randomized controlled trials. The CONSORT statement. *JAMA*. 1996;276(8):637-639.

2001

Moher D, Schulz KF, Altman DG; CONSORT. The CONSORT statement: revised recommendations for improving the quality of reports of parallel group randomized trials. BMC Med Res Methodol. 2001;1:2.

2010

Schulz KF, Altman DG, Moher D; CONSORT Group. CONSORT 2010 statement: updated guidelines for reporting parallel group randomised trials. BMJ. 2010 340:c332.

To promote transparency, clarity, and completeness

CONSORT 2010

Consolidated Standard for Reporting Trials

Checklist of 25 items

CONSORT Checklist

CONSORT 2010 checklist of information to include when reporting a randomised trial*

Section/Topic	Item No	Checklist item
Title and abstract		
	1a	Identification as a randomised trial in the title
	1b	Structured summary of trial design, methods, results, and conclusions (for specific guidance see CONSORT for abstracts [1, 2])
Introduction		
Background and objectives	2a	Scientific background and explanation of rationale
	2b	Specific objectives or hypotheses
Methods		
Trial design	3a	Description of trial design (such as parallel, factorial) including allocation ratio
	3b	Important changes to methods after trial commencement (such as eligibility criteria), with reasons
Participants	4a	Eligibility criteria for participants
	4b	Settings and locations where the data were collected
Interventions	5	The interventions for each group with sufficient details to allow replication, including how and when they were actually administered
Outcomes	6a	Completely defined pre-specified primary and secondary outcome measures, including how and when they were assessed
	6b	Any changes to trial outcomes after the trial commenced, with reasons
Sample size	7a	How sample size was determined
	7b	When applicable, explanation of any interim analyses and stopping guidelines
Randomisation:		
Sequence generation	8a	Method used to generate the random allocation sequence
	8b	Type of randomisation; details of any restriction (such as blocking and block size)
Allocation concealment mechanism	9	Mechanism used to implement the random allocation sequence (such as sequentially numbered containers), describing any steps taken to conceal the sequence until interventions were assigned
Implementation	10	Who generated the random allocation sequence, who enrolled participants, and who assigned participants to interventions
Blinding	11a	If done, who was blinded after assignment to interventions (for example, participants, care providers, those assessing outcomes) and how
	11b	If relevant, description of the similarity of interventions
Statistical methods	12a	Statistical methods used to compare groups for primary and secondary outcomes
	12b	Methods for additional analyses, such as subgroup analyses and adjusted analyses
Results		

https://www.equator-network.org/reporting-guidelines/consort/

CONSORT Checklist (continued)

Participant flow (a diagram is strongly recommended)	13a	For each group, the numbers of participants who were randomly assigned, received intended treatment, and were analysed for the primary outcome
	13b	For each group, losses and exclusions after randomisation, together with reasons
Recruitment	14a	Dates defining the periods of recruitment and follow-up
	14b	Why the trial ended or was stopped
Baseline data	15	A table showing baseline demographic and clinical characteristics for each group
Numbers analysed	16	For each group, number of participants (denominator) included in each analysis and whether the analysis was by original assigned groups
Outcomes and estimation	17a	For each primary and secondary outcome, results for each group, and the estimated effect size and its precision (such as 95% confidence interval)
	17b	For binary outcomes, presentation of both absolute and relative effect sizes is recommended
Ancillary analyses	18	Results of any other analyses performed, including subgroup analyses and adjusted analyses, distinguishing pre-specified from exploratory
Harms	19	All important harms or unintended effects in each group (for specific guidance see CONSORT for harms[28])
Discussion		
Limitations	20	Trial limitations, addressing sources of potential bias, imprecision, and, if relevant, multiplicity of analyses
Generalisability	21	Generalisability (external validity, applicability) of the trial findings
Interpretation	22	Interpretation consistent with results, balancing benefits and harms, and considering other relevant evidence
Other information		
Registration	23	Registration number and name of trial registry
Protocol	24	Where the full trial protocol can be accessed, if available
Funding	25	Sources of funding and other support (such as supply of drugs), role of funders

*We strongly recommend reading this statement in conjunction with the CONSORT 2010 Explanation and Elaboration [3] for important clarifications on all the items. If relevant, we also recommend reading CONSORT extensions for cluster randomised trials, [4] non-inferiority and equivalence trials [5], non-pharmacological treatments [6], herbal interventions [7], and pragmatic trials [8]. Additional extensions are forthcoming: for those and for up to date references relevant to this checklist, see www.consort-statement.org.

https://www.equator-network.org/reporting-guidelines/consort/

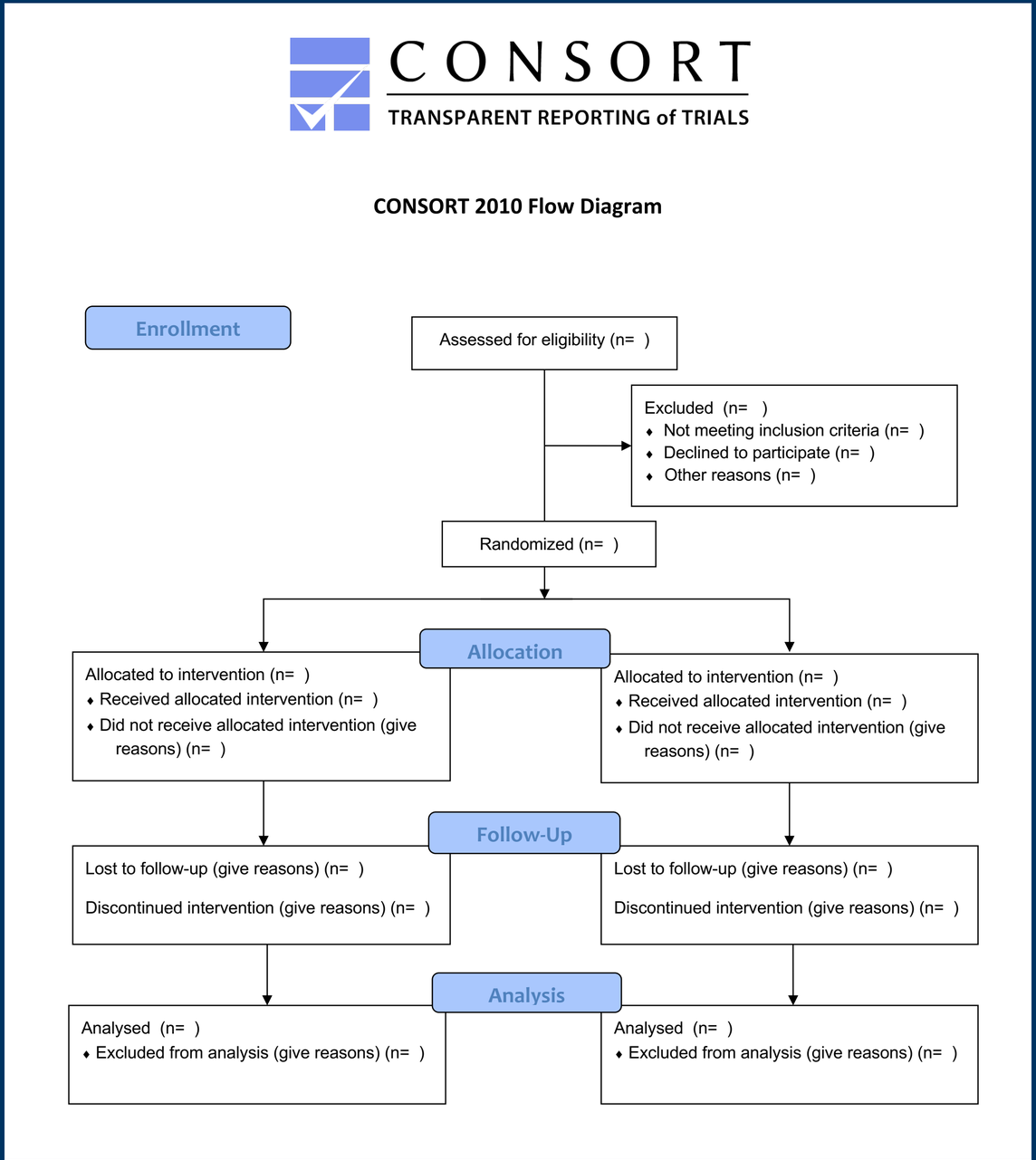
CONSORT 2010

Consolidated Standard for Reporting Trials

Checklist of 25 items

“Figure 1” - Consort Flowchart (items 13a and 13b)
“Table 1” - Items 15

CONSORT flowchart



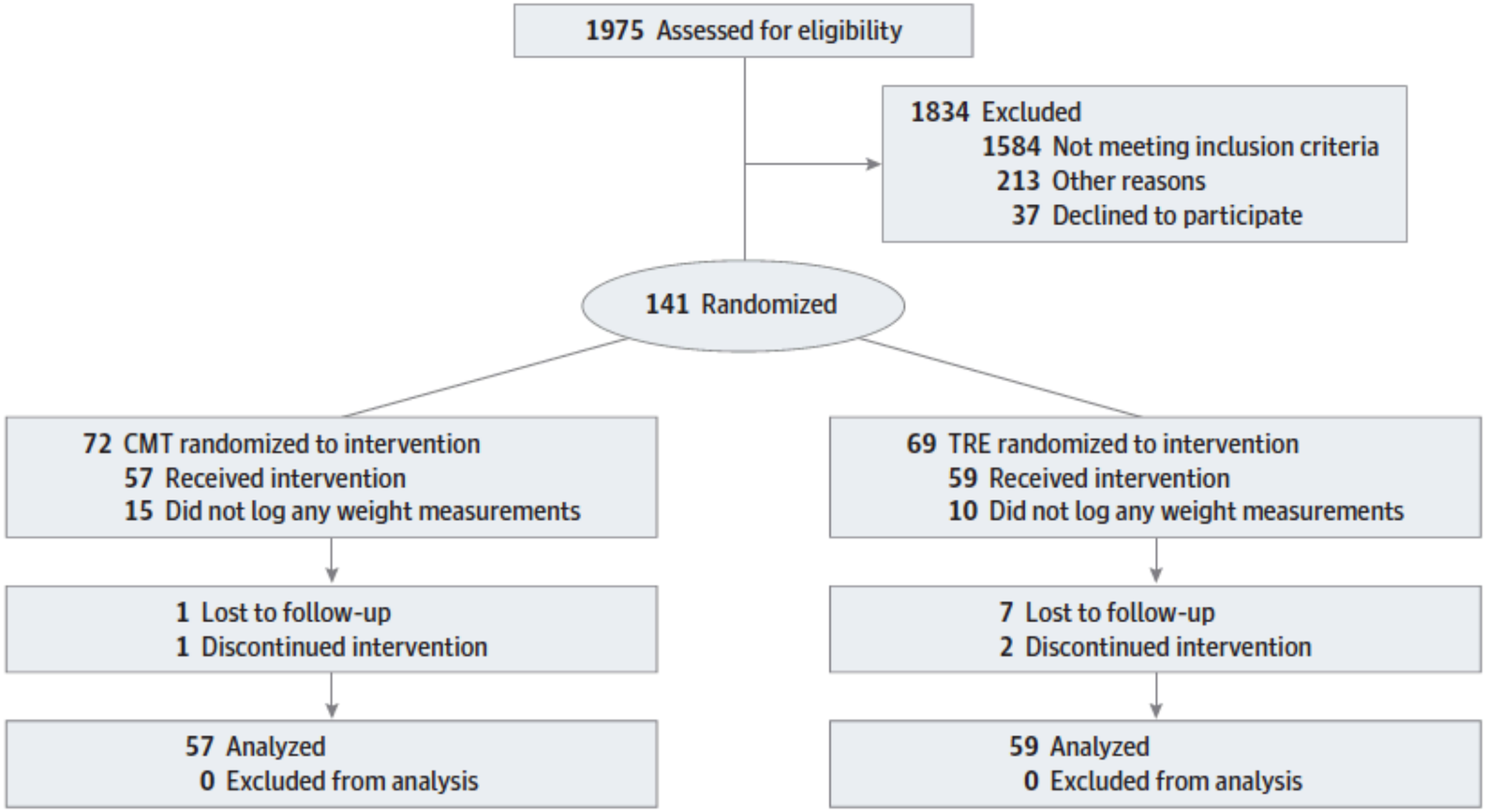
https://www.equator-network.org/reporting-guidelines/consort/

JAMA Internal Medicine | Original Investigation

Effects of Time-Restricted Eating on Weight Loss and Other Metabolic Parameters in Women and Men With Overweight and Obesity

The TREAT Randomized Clinical Trial

Dylan A. Lowe, PhD; Nancy Wu, MS; Linnea Rohdin-Bibby, BA; A. Holliston Moore, PhD; Nisa Kelly, MS; Yong En Liu, BS; Errol Philip, PhD; Eric Vittinghoff, PhD; Steven B. Heymsfield, MD; Jeffrey E. Olgin, MD; John A. Shepherd, PhD; Ethan J. Weiss, MD



What is important to show in the flowchart?

Article https://doi.org/10.1038/s41591-025-04181-w

Time-of-day immunochemotherapy in non-small cell lung cancer: a randomized phase 3 trial

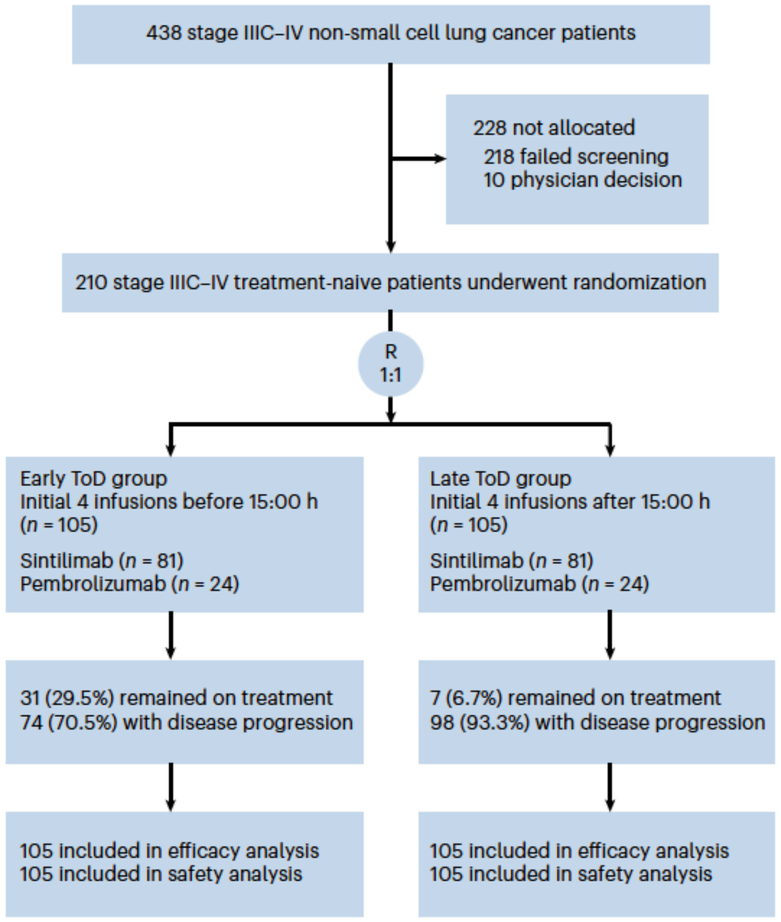


Fig. 1 | Flowchart of the present study. From September 2022 to May 2024, 210 patients were enrolled. Data cutoff time was 31 August 2025.

Table 1

What elements should be reported?

JAMA Internal Medicine | Original Investigation

Effects of Time-Restricted Eating on Weight Loss and Other Metabolic Parameters in Women and Men With Overweight and Obesity
The TREAT Randomized Clinical Trial

Dylan A. Lowe, PhD; Nancy Wu, MS; Linnea Rohdin-Bibby, BA; A. Holliston Moore, PhD; Nisa Kelly, MS; Yong En Liu, BS; Errol Philip, PhD; Eric Vittinghoff, PhD; Steven B. Heymsfield, MD; Jeffrey E. Olgin, MD; John A. Shepherd, PhD; Ethan J. Weiss, MD

Table 1. Baseline Characteristics

Characteristic	No. (%)		
	Total	CMT	TRE
Total cohort			
No.	116	57	59
Age, mean (SD), y	46.5 (10.5)	46.1 (10.3)	46.8 (10.8)
Female	46 (39.7)	22 (38.6)	24 (40.7)
Male	70 (60.3)	35 (61.4)	35 (59.3)
Weight, mean (SD), kg	99.2 (16.0)	99.1 (15.1)	99.3 (16.9)
BMI, mean (SD)	32.7 (4.2)	32.6 (3.4)	32.9 (4.9)
In-person cohort			
No.	50	25	25
Age, mean (SD), y	43.8 (11.2)	44.4 (10.7)	43.3(11.8)
Female	22 (44.0)	10 (40.0)	12 (48.0)
Male	28 (56.0)	15 (60.0)	13 (52.0)
Black	2 (4.0)	0	2 (8.0)
White	25 (50)	16 (64.0)	9 (36.0)
Latinx	7 (14.0)	3 (12.9)	4 (16.0)
Asian	12 (24.0)	5 (20.0)	7 (28.0)
Other/multi	4 (8.0)	1 (4.0)	3 (12.0)
Weight, mean (SD), kg	92.8 (14.2)	93.0 (13.3)	92.6 (15.2)
BMI, mean (SD),	31.4 (4.0)	31.3 (3.5)	31.5 (4.5)

Abbreviations: BMI, body mass index (calculated as weight in kilograms divided by height in meters squared); CMT, consistent meal timing group; TRE, time-restricted eating group.

Article https://doi.org/10.1038/s41591-025-04181-w

Time-of-day immunochemotherapy in non-small cell lung cancer: a randomized phase 3 trial

Table 1 | Baseline characteristics of patient

Characteristics	Early ToD group (*n*=105)	Late ToD group (*n*=105)	*P* value
Age (year, median, range)	61 (33–80)	60 (34–77)	0.53
Sex (*n*, %)			
Male	95 (90.5)	95 (90.5)	1.000
Female	10 (9.5)	10 (9.5)	
Smoking history (*n*, %)			
Smoker	84 (80.0)	87 (82.9)	0.590
Never smoked	21 (20.0)	18 (17.1)	
ECOG PS (*n*, %)			
0	38 (36.2)	32 (30.5)	0.380
1	67 (63.8)	73 (69.5)	
Histology (*n*, %)			
Squamous	60 (57.1)	56 (53.3)	0.580
Adenocarcinoma	45 (42.9)	49 (46.7)	
Stage (*n*, %)			
IIIC	24 (22.9)	17 (16.2)	0.120
IV	81 (77.1)	88 (83.8)	
Brain metastasis (*n*, %)			
Yes	13 (12.4)	16 (14.3)	0.550
No	92 (86.7)	89 (84.8)	
Liver metastasis (*n*, %)			
Yes	6 (5.7)	10 (9.5)	0.300
No	99 (94.3)	95 (90.5)	
PD-L1 TPS (*n*, %)			
<1%	41 (39.0%)	47 (44.8%)	0.862
1–49%	31 (29.5%)	29 (27.5%)	
≥50%	26 (24.8%)	22 (21.0%)	
Unknown	7 (6.7%)	7 (6.7%)	
ICIs (*n*, %)			
Sintilimab	81 (77.1%)	81 (77.1%)	1.000
Pembrolizumab	24 (22.9%)	24 (22.9%)	
Tumor burden (*n*, %)			
≥80 mm	56 (53.3)	43 (41.0)	0.097
<80 mm	49 (46.7)	62 (59.0)	
LDH (*n*, %)			
≤1 ULN	79 (75.2)	73 (69.5)	0.440
>1 ULN	26 (24.8)	32 (30.5)	
LIPI score (*n*, %)			
Low risk	63 (60.0)	58 (55.2)	0.673
Medium risk	35 (33.3)	37 (35.3)	
High risk	7 (6.7)	10 (9.5)	

Benefits of CONSORT 2010

Authors	Peer-reviewers/Editors	Readers
to assist reporting	to help in the revision process	to facilitate critical appraisal of the existing evidence

Limitations of CONSORT 2010

Encourage some authors to report fictitiously suggested by the guidance than what was actually done.

It does not include recommendations for designing and conducting a randomised trial

Some useful R packages for reporting

"Figure 1" CONSORT 2010 flowchart

visR (archived since last year)

ggconsort

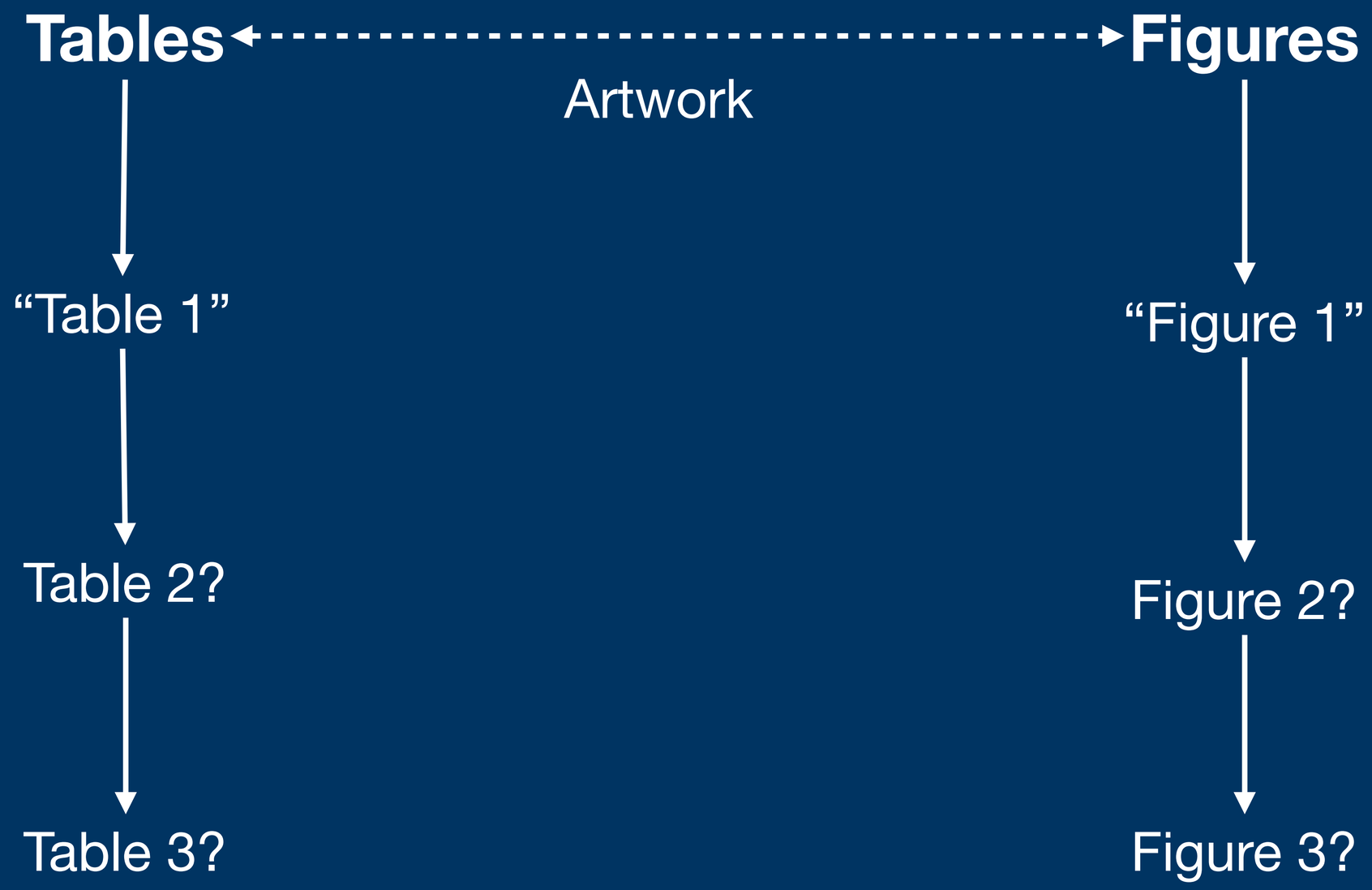
"Table 1" **(item 15 of CONSORT checklist)**

visR

Table1

Baseline demographic and clinical characteristics of study participants

Presenting the results



Presenting the results

Tables ⟷ **Figures**

Artwork

Tables		Figures
“Table 1”		“Figure 1”
Table 2?	Primary Endpoints	Figure 2?
Table 3?	Secondary Endpoints	Figure 3?

Presenting the results

Tables ⟷ **Figures**

Artwork

"Table 1" → Table 2? → Table 3?

"Figure 1" → Figure 2? → Figure 3?

A mix of art and science

Peer-reviewers

Editors

Group discussion

How many tables/figures?

When to use Tables?

Which Tables?

When to use Figures?

Which Figures?

Group discussion

How many tables/figures?

When to use Tables?

When to use Figures?

Which Tables?

Which Figures?

Storytelling/Understanding dara

Little quiz - what is the best plot?

Endpoint

Response/Non-Response

Disease score

Quantitative biomarker

Time-to-event

JAMA Internal Medicine | Original Investigation

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The TREAT Randomized Clinical Trial

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Primary endpoint:

change in weight loss

Secondary endpoints:

blood pressure, heart rate, total cholesterol, LDLc, HDLc, triglycerides, fasting glucose, fasting Insulin, C-Reactive Protein, Homocysteine

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3 Figures

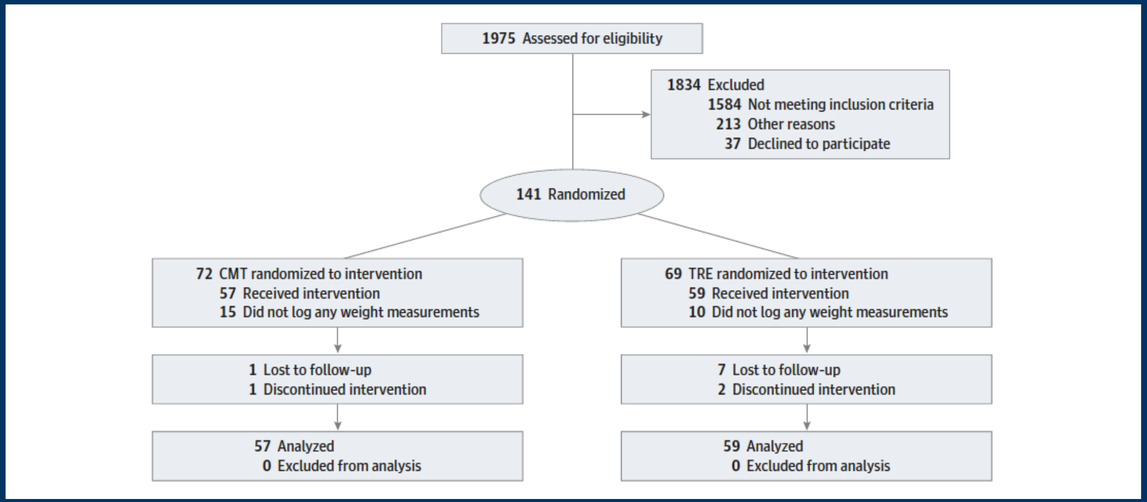


Figure 2. Prescribed vs Actual Energy Intake in the Alternate-Day Fasting and Daily Calorie Restriction Groups

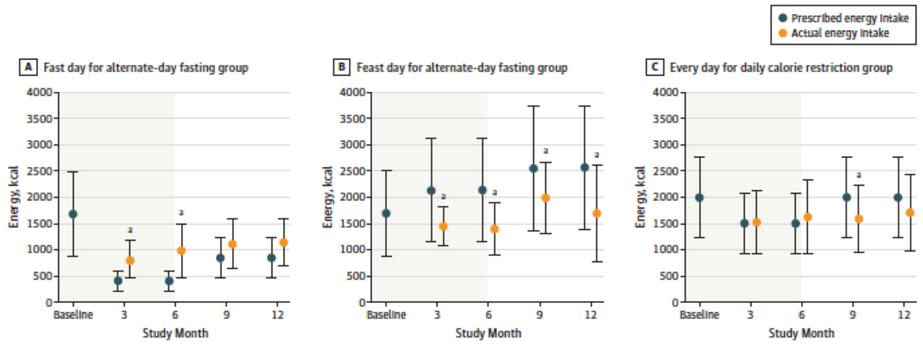
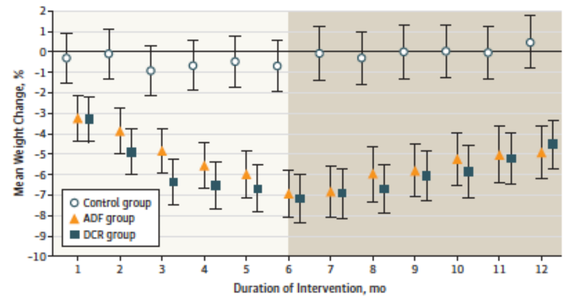


Figure 3. Weight Loss by Diet Group Relative to Baseline



Data were included for 100 participants; mean (SD) values were estimated using an intention-to-treat analysis with a linear mixed model. Error bars indicate 95% CIs for weight change from baseline by diet group at each time point (1-12 months). ADF indicates alternate-day fasting; DCR, daily calorie restriction.

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2 Tables

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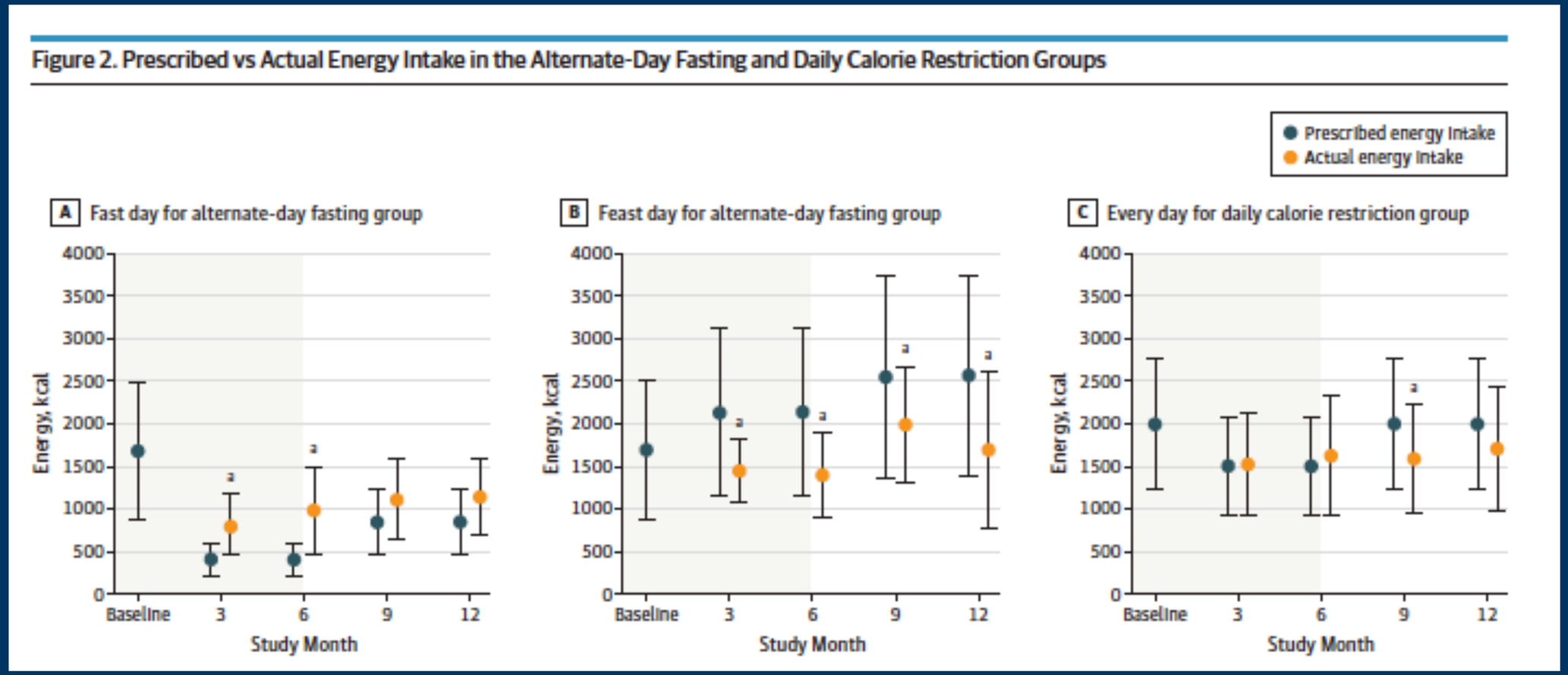
Abbreviations: BMI, body mass index (calculated as weight in kilograms divided by height in meters squared); CMT, consistent meal timing group; TRE, time-restricted eating group.

Table 2. Pairwise Effects Estimates of Diet on Mean Changes From Baseline in Body Weight and Risk Indicators for Cardiovascular Disease[a]

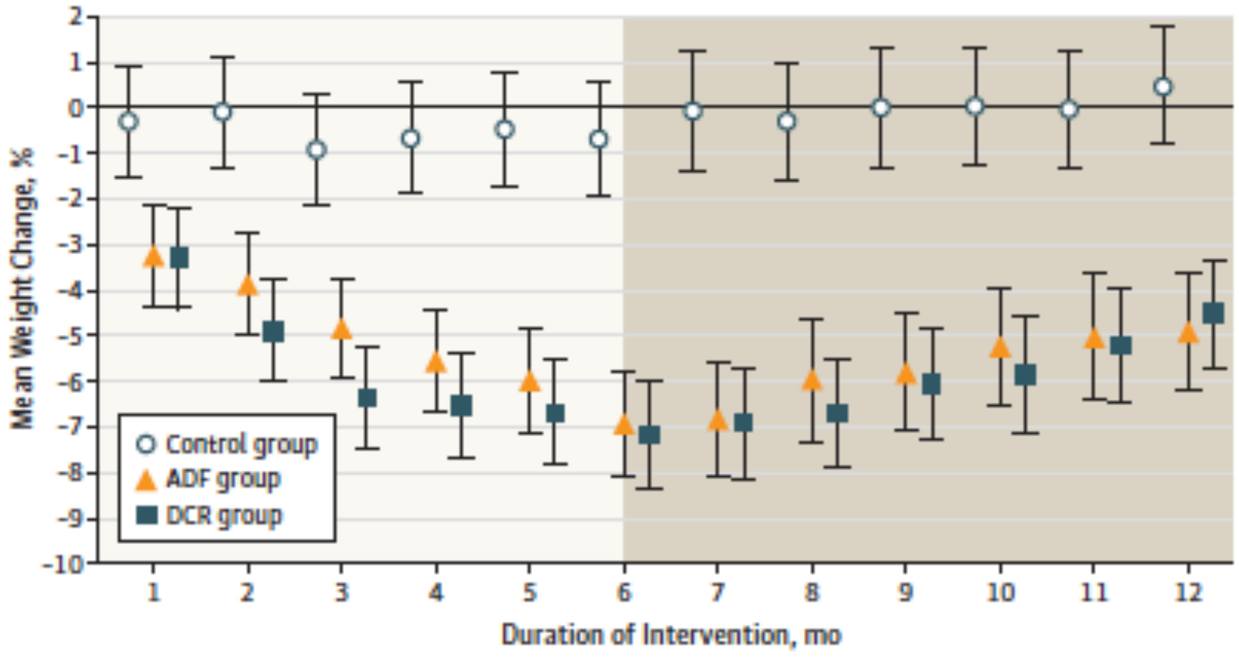
Outcome Variable	Change in ADF − Change in DCR (95% CI) At 6 mo	At 12 mo	Change in ADF − Change in Control (95% CI) At 6 mo	At 12 mo	Change in DCR − Change in Control (95% CI) At 6 mo	At 12 mo
Body weight, % change	0.0 (−2.2 to 2.2)	−0.7 (−3.1 to 1.6)	−6.8 (−9.1 to −4.5)	−6.0 (−8.5 to −3.6)	−6.8 (−9.1 to −4.6)	−5.3 (−7.6 to −3.0)
Fat mass, kg	0.9 (−1.3 to 3.1)	0.0 (−2.4 to 2.4)	−4.2 (−6.6 to −1.8)	−2.0 (−4.4 to 0.5)	−5.1 (−7.5 to −2.7)	−2.0 (−4.4 to 0.4)
Lean mass, kg	0.6 (−1.0 to 2.2)	0.5 (−1.2 to 2.2)	−1.5 (−3.2 to 0.2)	−0.9 (−2.7 to 0.9)	−2.1 (−3.8 to 0.4)	−1.4 (−3.1 to 0.3)
Visceral fat mass, kg	0.2 (−0.1 to 0.5)	0.1 (−0.2 to 0.5)	−0.4 (−0.7 to −0.1)	−0.4 (−0.7 to −0.1)	−0.6 (−0.9 to −0.2)	−0.5 (−0.8 to −0.2)
Blood pressure, mm Hg						
Systolic	0.8 (−7.1 to 8.7)	−1.1 (−9.5 to 7.4)	−3.1 (−11.3 to 5.2)	−2.3 (−11.0 to 6.4)	−3.9 (−12.1 to 4.4)	−1.24 (−9.7 to 7.2)
Diastolic	−0.3 (−5.9 to 5.4)	−3.0 (−9.0 to 3.0)	−1.5 (−7.4 to 4.4)	−0.1 (−6.3 to 6.1)	−1.2 (−7.1 to 4.6)	2.9 (−3.1 to 8.9)
Heart rate, beats/min	−4.9 (−10.1 to 0.4)	−2.0 (−7.7 to 3.8)	−5.8 (−11.3 to −0.3)	−1.2 (−7.1 to 4.7)	−0.9 (−6.4 to 4.5)	0.8 (−4.8 to 6.4)
Cholesterol, mg/dL						
Total	3.4 (−7.2 to 13.9)	9.7 (−2.2 to 21.7)	−4.3 (−15.4 to 6.9)	4.2 (−8.2 to 16.5)	−7.6 (−18.8 to 3.6)	−5.6 (−17.6 to 6.4)
HDL	6.2 (0.1 to 12.4)	1.0 (−5.9 to 7.8)	8.4 (1.9 to 14.7)	2.9 (−4.2 to 10.0)	2.2 (−4.3 to 8.7)	1.9 (−5.1 to 8.9)
LDL	2.5 (−6.0 to 10.9)	11.5 (1.9 to 21.1)	−2.6 (−11.5 to 6.4)	1.2 (−8.7 to 11.2)	−5.0 (−14.0 to 3.9)	−10.3 (−19.9 to −0.6)
Triglycerides, mg/dL	−10.5 (−26.7 to 5.8)	−9.9 (−28.3 to 8.6)	−19.1 (−36.3 to −1.8)	−24.4 (−43.5 to −5.3)	−8.6 (−25.9 to 8.7)	−14.5 (−33.1 to 4.0)
Glucose, mg/dL	−1.4 (−8.0 to 5.2)	5.7 (−1.6 to 13.0)	−6.3 (−13.3 to 0.7)	−3.9 (−11.5 to 3.6)	−4.9 (−12.0 to 2.1)	−9.6 (−17.1 to −2.2)
Insulin, μIU/mL	−0.4 (−5.5 to 4.7)	−1.3 (−6.9 to 4.3)	−7.5 (−12.9 to −2.0)	−5.9 (−11.7 to −0.1)	−7.0 (−12.5 to −1.6)	−4.6 (−10.4 to 1.2)
HOMA-IR[a]	0.07 (−1.56 to 1.70)	0.02 (−1.78 to 1.81)	−2.49 (−4.22 to −0.76)	−1.86 (−3.73 to 0.01)	−2.56 (−4.30 to −0.82)	−1.88 (−3.72 to −0.03)
HS CRP, mg/dL	−0.04 (−0.19 to 0.11)	0.00 (−0.16 to 0.17)	−0.07 (−0.23 to 0.08)	−0.07 (−0.24 to 0.11)	−0.04 (−0.19 to 0.12)	−0.07 (−0.24 to 0.10)
Homocysteine, mg/L	0.03 (−0.10 to 0.17)	0.03 (−0.12 to 0.18)	0.10 (−0.04 to 0.24)	0.02 (−0.13 to 0.18)	0.06 (−0.08 to 0.20)	−0.01 (−0.16 to 0.14)

What does this figure tell us?



What does this figure tell us?

Figure 3. Weight Loss by Diet Group Relative to Baseline



Data were included for 100 participants; mean (SD) values were estimated using an intention-to-treat analysis with a linear mixed model. Error bars indicate 95% CIs for weight change from baseline by diet group at each time point (1-12 months). ADF indicates alternate-day fasting; DCR, daily calorie restriction.

What does this table tell us?

Table 2. Pairwise Effects Estimates of Diet on Mean Changes From Baseline in Body Weight and Risk Indicators for Cardiovascular Disease[a]

	Change in ADF − Change in DCR (95% CI)		Change in ADF − Change in Control (95% CI)		Change in DCR − Change in Control (95% CI)	
Outcome Variable	At 6 mo	At 12 mo	At 6 mo	At 12 mo	At 6 mo	At 12 mo
Body weight, % change	0.0 (−2.2 to 2.2)	−0.7 (−3.1 to 1.6)	−6.8 (−9.1 to −4.5)	−6.0 (−8.5 to −3.6)	−6.8 (−9.1 to −4.6)	−5.3 (−7.6 to −3.0)
Fat mass, kg	0.9 (−1.3 to 3.1)	0.0 (−2.4 to 2.4)	−4.2 (−6.6 to −1.8)	−2.0 (−4.4 to 0.5)	−5.1 (−7.5 to −2.7)	−2.0 (−4.4 to 0.4)
Lean mass, kg	0.6 (−1.0 to 2.2)	0.5 (−1.2 to 2.2)	−1.5 (−3.2 to 0.2)	−0.9 (−2.7 to 0.9)	−2.1 (−3.8 to 0.4)	−1.4 (−3.1 to 0.3)
Visceral fat mass, kg	0.2 (−0.1 to 0.5)	0.1 (−0.2 to 0.5)	−0.4 (−0.7 to −0.1)	−0.4 (−0.7 to −0.1)	−0.6 (−0.9 to −0.2)	−0.5 (−0.8 to −0.2)
Blood pressure, mm Hg						
Systolic	0.8 (−7.1 to 8.7)	−1.1 (−9.5 to 7.4)	−3.1 (−11.3 to 5.2)	−2.3 (−11.0 to 6.4)	−3.9 (−12.1 to 4.4)	−1.24 (−9.7 to 7.2)
Diastolic	−0.3 (−5.9 to 5.4)	−3.0 (−9.0 to 3.0)	−1.5 (−7.4 to 4.4)	−0.1 (−6.3 to 6.1)	−1.2 (−7.1 to 4.6)	2.9 (−3.1 to 8.9)
Heart rate, beats/min	−4.9 (−10.1 to 0.4)	−2.0 (−7.7 to 3.8)	−5.8 (−11.3 to −0.3)	−1.2 (−7.1 to 4.7)	−0.9 (−6.4 to 4.5)	0.8 (−4.8 to 6.4)
Cholesterol, mg/dL						
Total	3.4 (−7.2 to 13.9)	9.7 (−2.2 to 21.7)	−4.3 (−15.4 to 6.9)	4.2 (−8.2 to 16.5)	−7.6 (−18.8 to 3.6)	−5.6 (−17.6 to 6.4)
HDL	6.2 (0.1 to 12.4)	1.0 (−5.9 to 7.8)	8.4 (1.9 to 14.7)	2.9 (−4.2 to 10.0)	2.2 (−4.3 to 8.7)	1.9 (−5.1 to 8.9)
LDL	2.5 (−6.0 to 10.9)	11.5 (1.9 to 21.1)	−2.6 (−11.5 to 6.4)	1.2 (−8.7 to 11.2)	−5.0 (−14.0 to 3.9)	−10.3 (−19.9 to −0.6)
Triglycerides, mg/dL	−10.5 (−26.7 to 5.8)	−9.9 (−28.3 to 8.6)	−19.1 (−36.3 to −1.8)	−24.4 (−43.5 to −5.3)	−8.6 (−25.9 to 8.7)	−14.5 (−33.1 to 4.0)
Glucose, mg/dL	−1.4 (−8.0 to 5.2)	5.7 (−1.6 to 13.0)	−6.3 (−13.3 to 0.7)	−3.9 (−11.5 to 3.6)	−4.9 (−12.0 to 2.1)	−9.6 (−17.1 to −2.2)
Insulin, μIU/mL	−0.4 (−5.5 to 4.7)	−1.3 (−6.9 to 4.3)	−7.5 (−12.9 to −2.0)	−5.9 (−11.7 to −0.1)	−7.0 (−12.5 to −1.6)	−4.6 (−10.4 to 1.2)
HOMA-IR[a]	0.07 (−1.56 to 1.70)	0.02 (−1.78 to 1.81)	−2.49 (−4.22 to −0.76)	−1.86 (−3.73 to 0.01)	−2.56 (−4.30 to −0.82)	−1.88 (−3.72 to −0.03)
HS CRP, mg/dL	−0.04 (−0.19 to 0.11)	0.00 (−0.16 to 0.17)	−0.07 (−0.23 to 0.08)	−0.07 (−0.24 to 0.11)	−0.04 (−0.19 to 0.12)	−0.07 (−0.24 to 0.10)
Homocysteine, mg/L	0.03 (−0.10 to 0.17)	0.03 (−0.12 to 0.18)	0.10 (−0.04 to 0.24)	0.02 (−0.13 to 0.18)	0.06 (−0.08 to 0.20)	−0.01 (−0.16 to 0.14)

Article https://doi.org/10.1038/s41591-025-04181-w

Time-of-day immunochemotherapy in non-small cell lung cancer: a randomized phase 3 trial

3 Figures (+ 1 Figure)

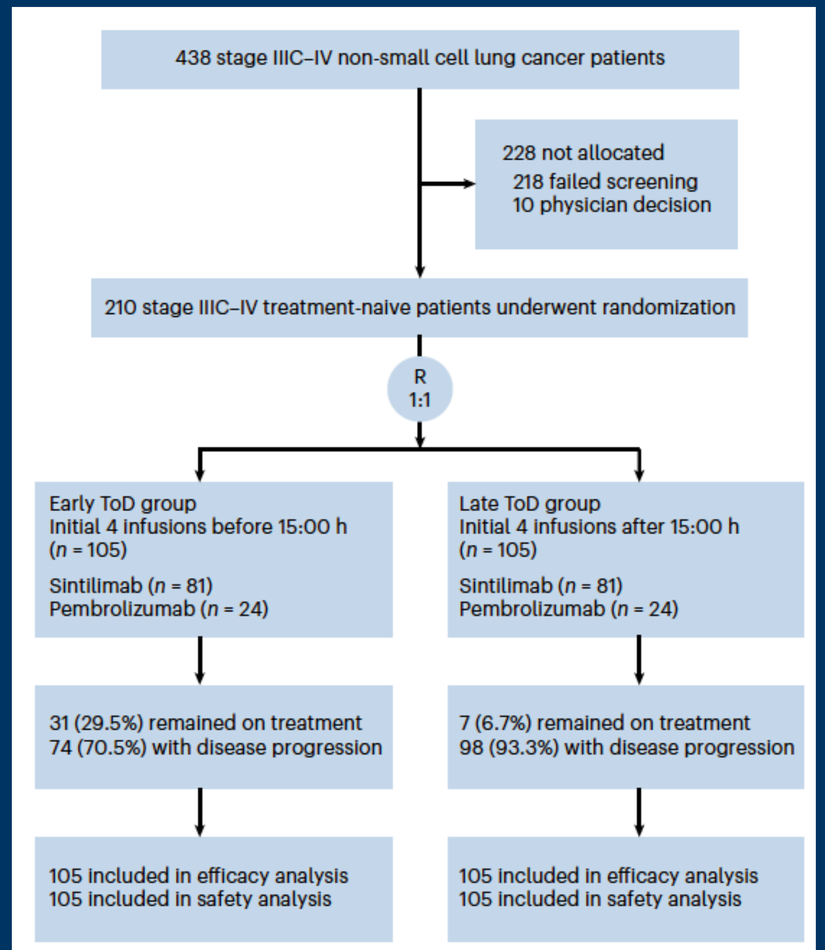
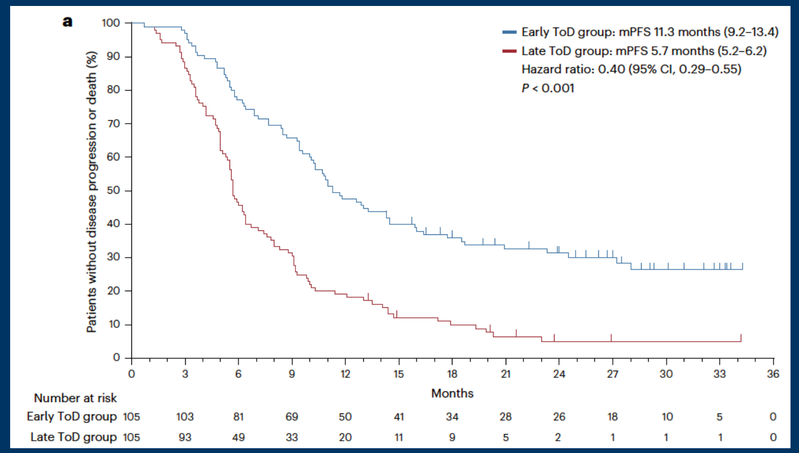
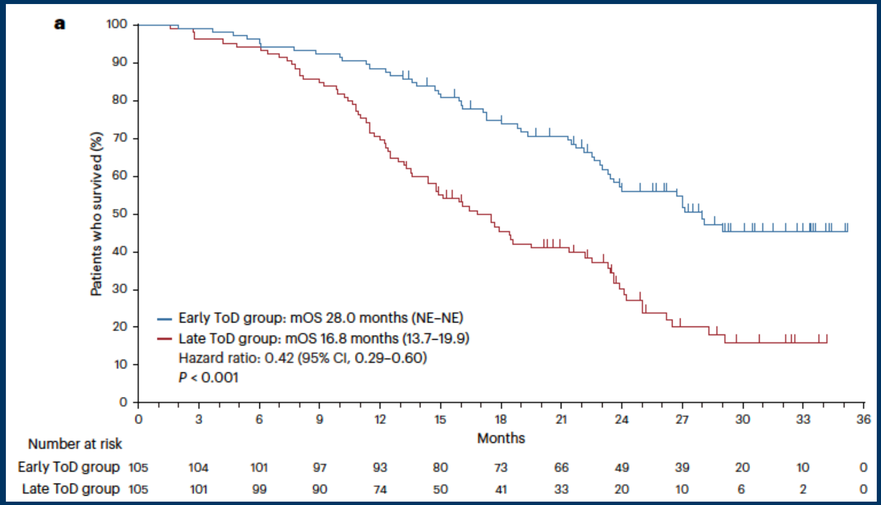


Fig. 1 | Flowchart of the present study. From September 2022 to May 2024, 210 patients were enrolled. Data cutoff time was 31 August 2025.





Article

https://doi.org/10.1038/s41591-025-04181-w

Time-of-day immunochemotherapy in non-small cell lung cancer: a randomized phase 3 trial

Primary endpoint (efficacy):

Disease Progression

Secondary endpoints (efficacy):

Overall survival

Article https://doi.org/10.1038/s41591-025-04181-w

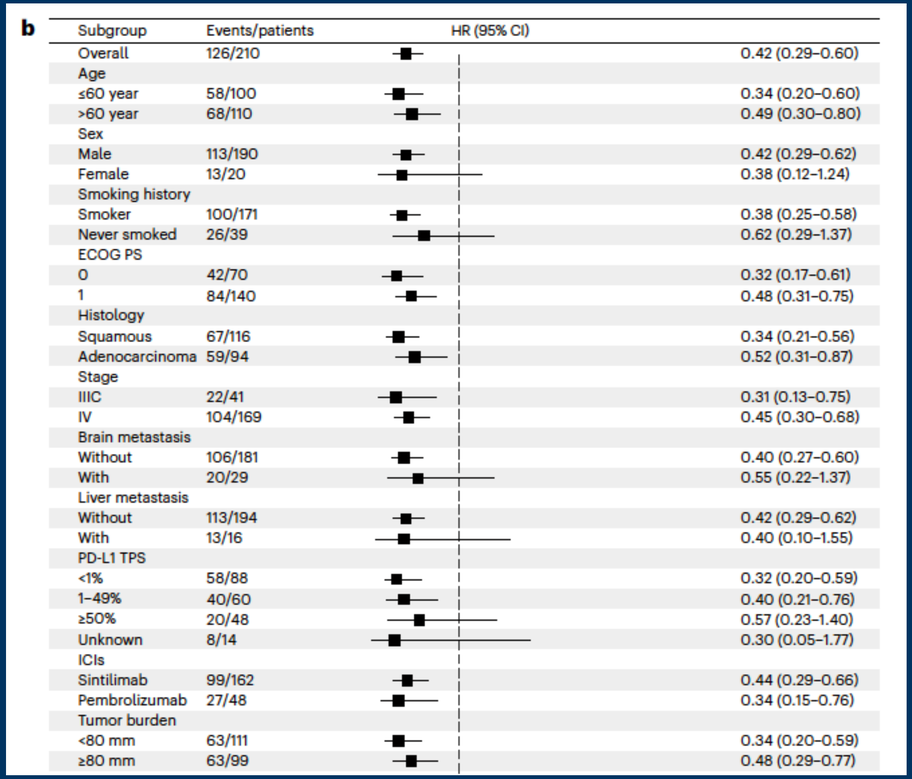
Time-of-day immunochemotherapy in non-small cell lung cancer: a randomized phase 3 trial

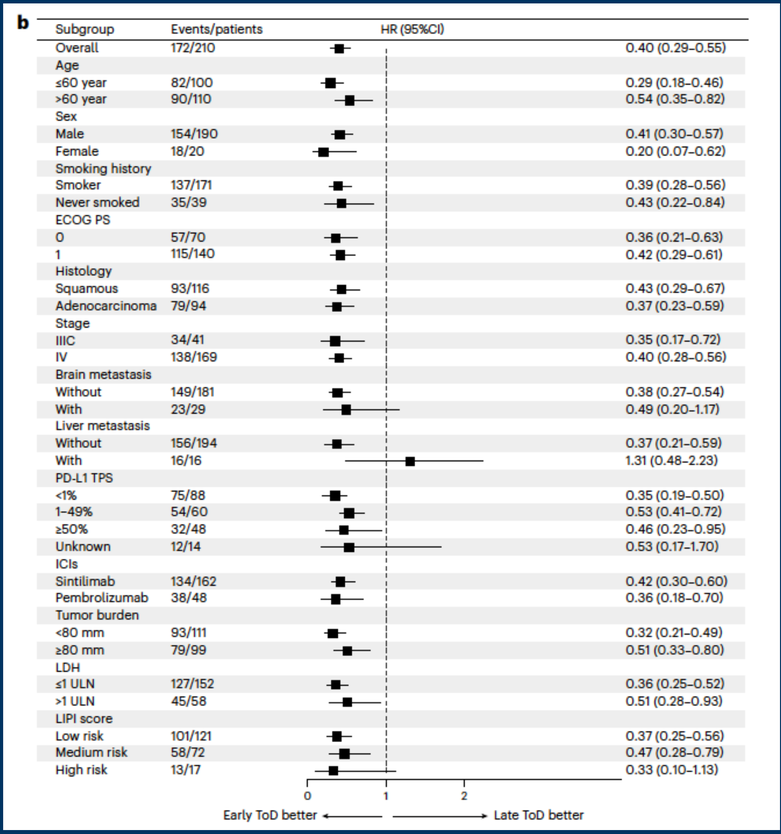
3 Tables

"Table 1"

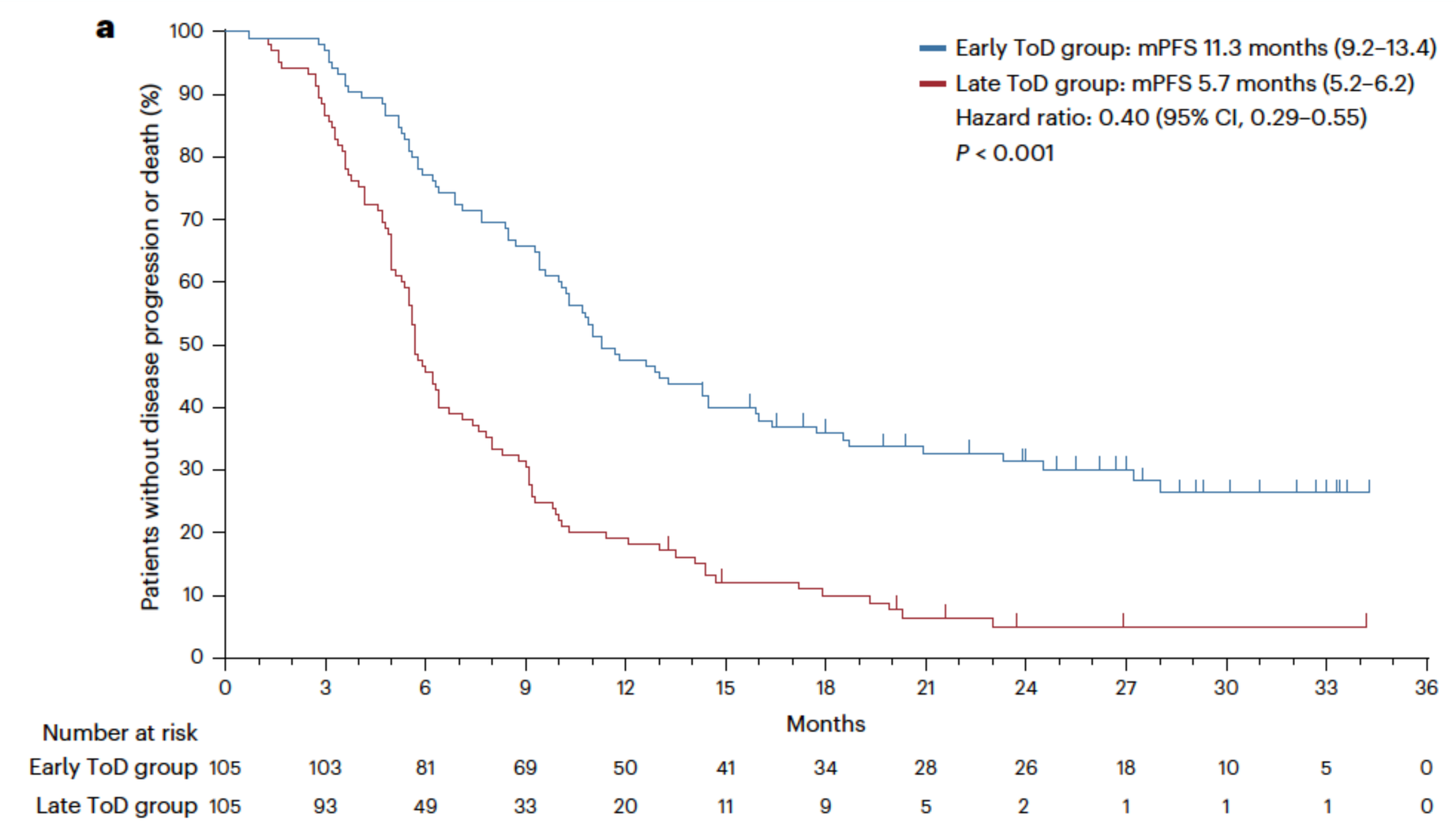
Brain metastasis (*n*, %)			
Yes	13 (12.4)	16 (14.3)	0.550
No	92 (86.7)	89 (84.8)	
Liver metastasis (*n*, %)			
Yes	6 (5.7)	10 (9.5)	0.300
No	99 (94.3)	95 (90.5)	
PD-L1 TPS (*n*, %)			
<1%	41 (39.0%)	47 (44.8%)	0.862
1–49%	31 (29.5%)	29 (27.5%)	
≥50%	26 (24.8%)	22 (21.0%)	
Unknown	7 (6.7%)	7 (6.7%)	
ICIs (*n*, %)			
Sintilimab	81 (77.1%)	81 (77.1%)	1.000
Pembrolizumab	24 (22.9%)	24 (22.9%)	
Tumor burden (*n*, %)			
≥80 mm	56 (53.3)	43 (41.0)	0.097
<80 mm	49 (46.7)	62 (59.0)	
LDH (*n*, %)			
≤1 ULN	79 (75.2)	73 (69.5)	0.440
>1 ULN	26 (24.8)	32 (30.5)	
LIPI score (*n*, %)			
Low risk	63 (60.0)	58 (55.2)	0.673
Medium risk	35 (33.3)	37 (35.3)	
High risk	7 (6.7)	10 (9.5)	

Actually they were included in Figures

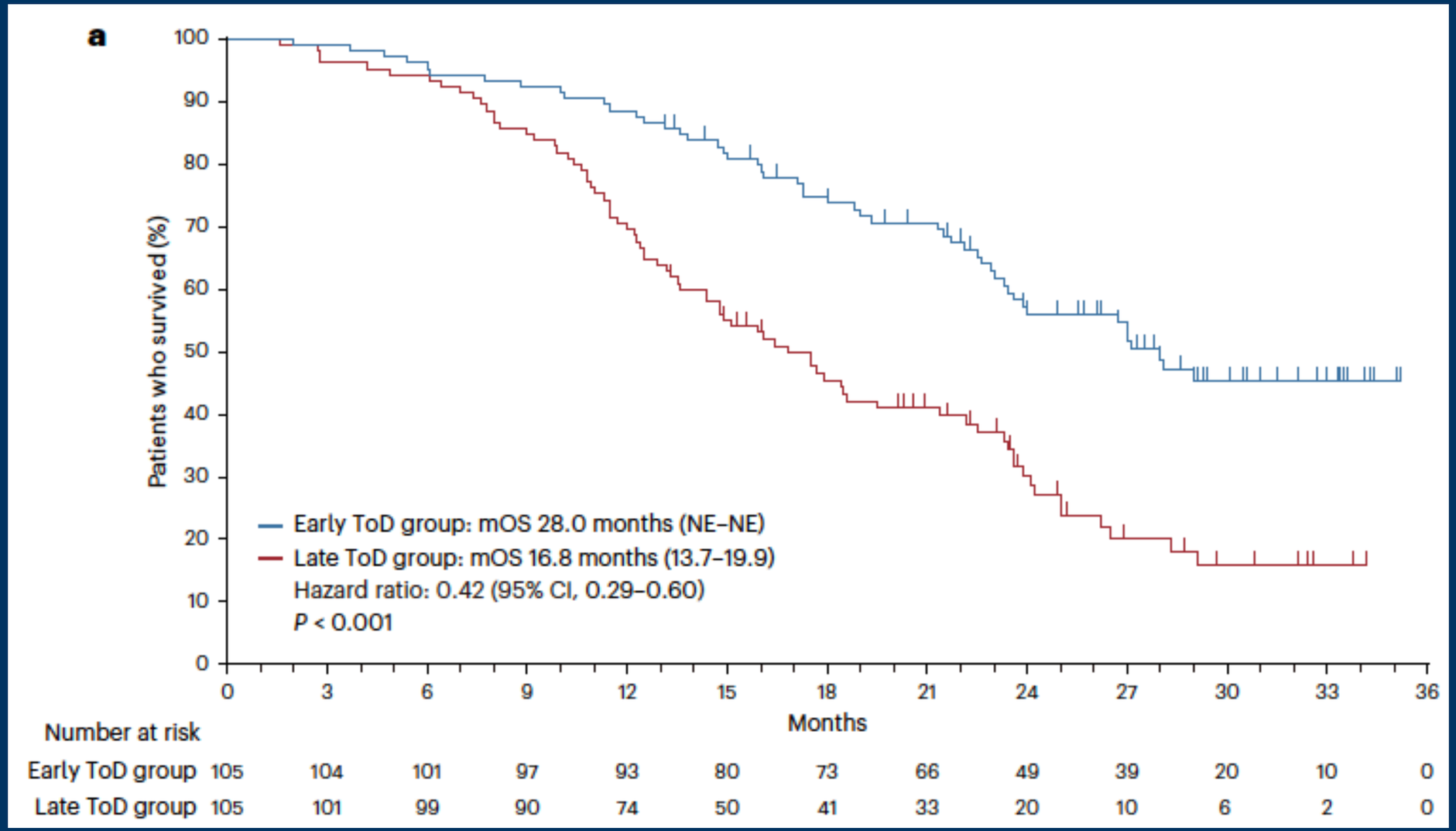




What does this Figure tell you?



What does this Figure tell you?



Spotting strange things

Table 1 | Baseline characteristics of patient

Characteristics	Early ToD group (*n*=105)	Late ToD group (*n*=105)	*P* value
Age (year, median, range)	61 (33–80)	60 (34–77)	0.53
Sex (*n*, %)			
Male	95 (90.5)	95 (90.5)	1.000
Female	10 (9.5)	10 (9.5)	
Smoking history (*n*, %)			
Smoker	84 (80.0)	87 (82.9)	0.590
Never smoked	21 (20.0)	18 (17.1)	
ECOG PS (*n*, %)			
0	38 (36.2)	32 (30.5)	0.380
1	67 (63.8)	73 (69.5)	
Histology (*n*, %)			
Squamous	60 (57.1)	56 (53.3)	0.580
Adenocarcinoma	45 (42.9)	49 (46.7)	
Stage (*n*, %)			
IIIC	24 (22.9)	17 (16.2)	0.120
IV	81 (77.1)	88 (83.8)	
Brain metastasis (*n*, %)			
Yes	13 (12.4)	16 (14.3)	0.550
No	92 (86.7)	89 (84.8)	
Liver metastasis (*n*, %)			
Yes	6 (5.7)	10 (9.5)	0.300
No	99 (94.3)	95 (90.5)	
PD-L1 TPS (*n*, %)			
<1%	41 (39.0%)	47 (44.8%)	0.862
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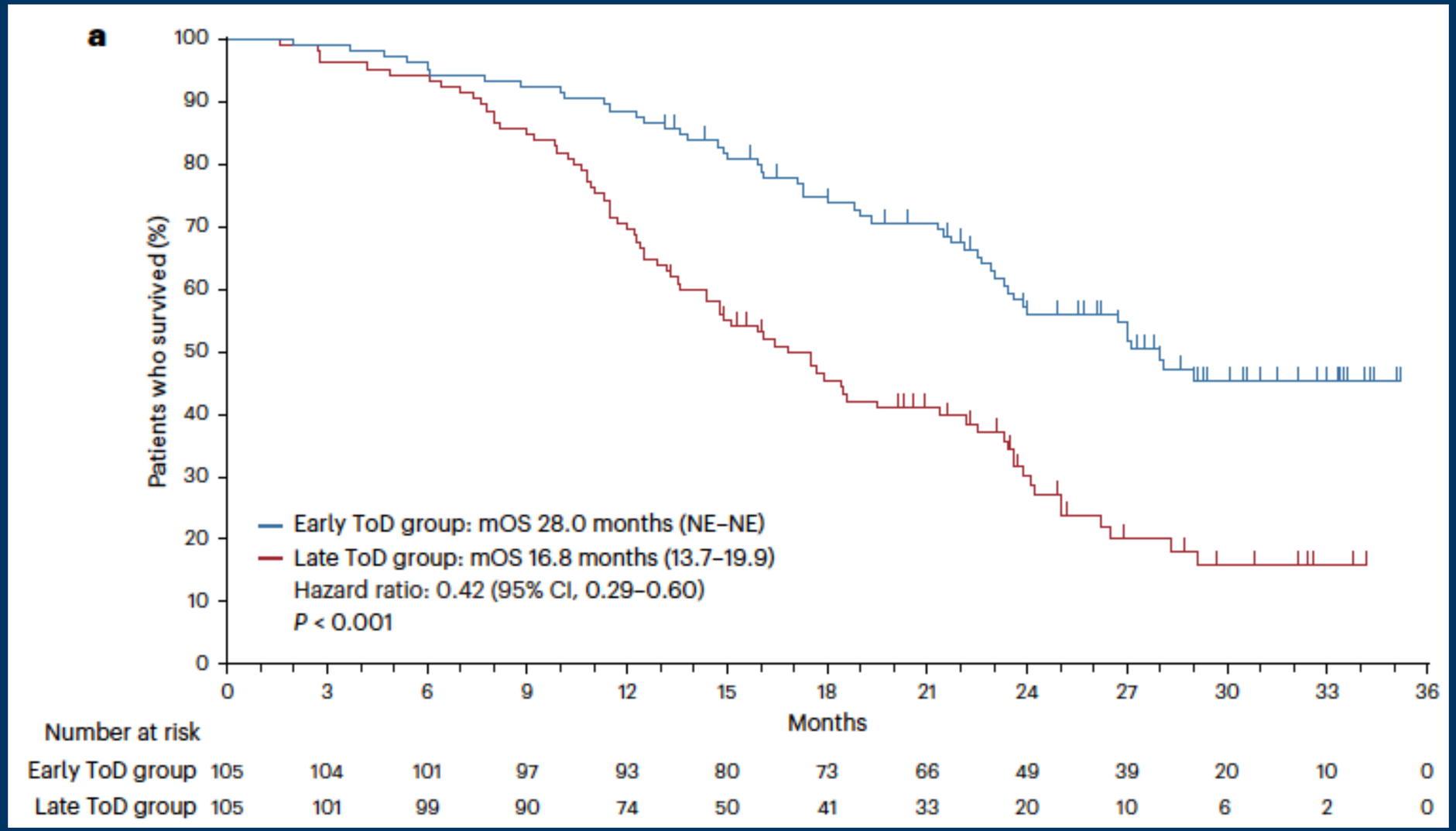
Spotting strange things



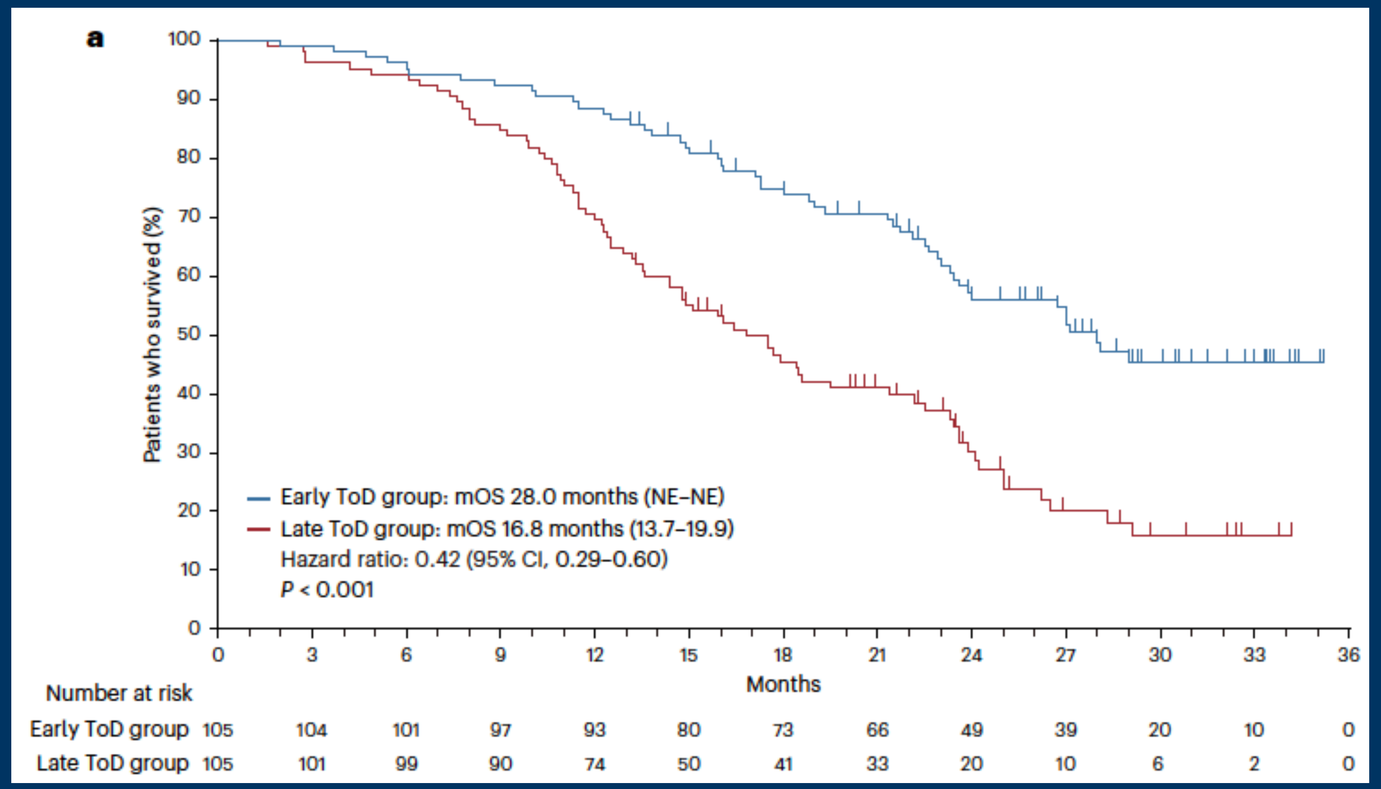
Spotting strange things

In the clinical trial registration

Inclusion Criteria:

1. Understand the requirements and contents of the clinical trial, and provide a signed and dated informed consent form.
2. Age ≥ 18 years.
3. Histopathology or cytology confirmed and recorded local progression or metastatic non-small cell lung cancer without systemic treatment.
4. EGFR, ALK and ROS1 mutations negative confirmed by an accredited local laboratory.
5. ECOG 0-1.
6. Predicted survival ≥ 12 weeks.
7. Adequate bone marrow hematopoiesis and organ function
8. Presence of measurable lesions according to RECIST 1.1.
9. Subjects with stable brain metastases may be included in the study.

Spotting strange things



What's wrong with this data?

Spotting strange things

Article

https://doi.org/10.1038/s41591-025-04181-w

Time-of-day immunochemotherapy in non-small cell lung cancer: a randomized phase 3 trial

What's wrong with this title?

Study design

This prospective, randomized, single-center, open-label phase 3 trial (LungTIME-C01) was designed to compare survival outcomes between patients who received their first four cycles of immunochemotherapy before 15:00 h and those who received them after 15:00 h. Participants were enrolled at Hunan Cancer Hospital between 22 September 2022 and 21 May 2024.

Spotting strange things